

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 01	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 01

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. Carlos, a 51-year-old university student, presents with recurrent unexpected panic attacks followed by persistent worry about more attacks or maladaptive avoidance. Which diagnosis or clinical concept best fits this presentation?

- A. Obsessive-compulsive personality disorder (OCPD)
- B. Bipolar disorder
- C. Brief psychotic disorder

D. Panic disorder

Answer: D - Panic disorder. Panic disorder is more than a single panic attack: recurrent unexpected attacks are followed by ongoing concern or behavior change.

Memory aid: PANIC: Panic Attacks + New worry/avoidance Is Continuing.

2. During assessment, Yvette reports excessive, difficult-to-control worry across several life areas for about six months, with tension, restlessness or poor sleep. Which of the following is the BEST match?

- A. Major depressive disorder
- B. Bulimia nervosa
- C. Generalized anxiety disorder (GAD)**
- D. Agoraphobia

Answer: C - Generalized anxiety disorder (GAD). GAD is characterized by persistent generalized worry across multiple domains rather than fear tied to one object or situation.

Memory aid: GAD = General Anxiety across Domains.

3. Xavier, a 45-year-old sales representative, presents with marked fear of a specific object or situation that triggers immediate anxiety and avoidance out of proportion to the actual danger. Which diagnosis or clinical concept best fits this presentation?

- A. Specific phobia**
- B. Agoraphobia
- C. Dependent personality disorder
- D. Substance use disorder

Answer: A - Specific phobia. A specific phobia is circumscribed: the fear is linked to a particular object or situation and typically produces avoidance.

Memory aid: SPECIFIC = one SPECIFIC trigger.

4. A therapist notes that Owen has marked fear of social or performance situations because of possible scrutiny, embarrassment or negative evaluation. Which condition should be considered first from the options below?

- A. Specific phobia

B. Obsessive-compulsive disorder (OCD)

C. Social anxiety disorder

D. Agoraphobia

Answer: C - Social anxiety disorder. Social anxiety centers on being observed, judged or embarrassed in social/performance settings.

Memory aid: SOCIAL = *Scrutiny Of Client In Activities with Lots of people.*

5. Quentin, a 25-year-old nurse, presents with fear and avoidance of situations where escape may be difficult or help unavailable, such as crowds, public transport or being outside alone. Which diagnosis or clinical concept best fits this presentation?

A. Agoraphobia

B. Bipolar disorder

C. Autism spectrum disorder

D. Social anxiety disorder

Answer: A - Agoraphobia. Agoraphobia concerns situations in which escape or access to help feels difficult; it may occur with or without panic disorder.

Memory aid: AGORA = *Afraid to Go Out/Risk being Away from help.*

6. A therapist notes that Uma has intrusive unwanted thoughts or urges and/or repetitive behaviors or mental acts performed to reduce anxiety. Which condition should be considered first from the options below?

A. Obsessive-compulsive personality disorder (OCPD)

B. Bipolar disorder

C. Obsessive-compulsive disorder (OCD)

D. Borderline personality disorder

Answer: C - Obsessive-compulsive disorder (OCD). OCD involves obsessions (intrusive thoughts/urges/images) and compulsions (repetitive acts or mental rituals).

Memory aid: O = *Obsession is a thought; C = Compulsion is a Conduct.*

7. A therapist notes that Irene has after trauma, recurrent intrusions or nightmares, avoidance, negative changes in mood/cognition and heightened arousal persisting beyond one month. Which condition should be considered first from the options below?

A. Bulimia nervosa

B. Post-traumatic stress disorder (PTSD)

C. Brief psychotic disorder

D. Specific phobia

Answer: B - Post-traumatic stress disorder (PTSD). PTSD follows exposure to trauma and is organized around intrusion, avoidance, negative cognitive/mood changes and arousal/reactivity.

Memory aid: IANA: *Intrusion, Avoidance, Negative changes, Arousal.*

8. During assessment, Uma reports at least two weeks of depressed mood and/or loss of interest with associated sleep, appetite, energy, concentration, guilt, psychomotor or suicidal symptoms. Which of the following is the BEST match?

A. Major depressive disorder

B. Schizoid personality disorder

C. Binge-eating disorder

D. Post-traumatic stress disorder (PTSD)

Answer: A - Major depressive disorder. Major depression is a sustained depressive syndrome, not simply transient sadness. Functional impairment and risk, especially suicidality, must be assessed.

Memory aid: SIGECAPS: *Sleep, Interest, Guilt, Energy, Concentration, Appetite, Psychomotor, Suicide.*

9. Yvette, a 42-year-old university student, presents with abnormally elevated or irritable mood with increased energy, reduced need for sleep, pressured speech, grandiosity, racing thoughts and risky activity. Which diagnosis or clinical concept best fits this presentation?

A. Major depressive disorder

B. Autism spectrum disorder

C. Stimulant intoxication

D. Manic episode

Answer: D - Manic episode. A manic episode combines abnormal mood elevation/irritability with increased energy and characteristic activation symptoms.

Memory aid: DIG FAST: Distractible, Indiscretion, Grandiosity, Flight of ideas, Activity, Sleep less, Talkative.

10. During assessment, Patricia reports a history of a manic episode, often alternating with periods of depression. Which of the following is the BEST match?

- A. Dependent personality disorder
- B. Agoraphobia
- C. Bipolar disorder**
- D. Specific phobia

Answer: C - Bipolar disorder. The defining feature of bipolar I disorder is a manic episode; depressive episodes are common but not required for that diagnosis.

Memory aid: BI-polar = mood can move toward two poles; mania is the key flag.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. A panic attack can occur in disorders other than panic disorder.

A panic attack is a symptom episode and can occur across several disorders; panic disorder requires a recurrent pattern plus ongoing concern or behavior change.

2. FALSE. Obsessions are repetitive behaviors, whereas compulsions are intrusive thoughts.

The relationship is reversed: obsessions are intrusive thoughts/urges/images; compulsions are repetitive behaviors or mental acts.

3. TRUE. Trauma exposure can increase risk for depression, anxiety and substance-related problems.

Trauma can be followed by several forms of psychopathology, not only PTSD.

4. FALSE. A single laboratory test confirms schizophrenia.

Schizophrenia is diagnosed clinically from symptoms, course, duration and impairment while ruling out other causes.

5. TRUE. In major depression, loss of interest or pleasure is called anhedonia.

Anhedonia is a core depressive symptom.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate a panic attack from panic disorder. (2mks)

- A panic attack is an acute episode of intense fear/discomfort with physical/cognitive symptoms.
- Panic disorder requires recurrent unexpected attacks plus persistent concern and/or maladaptive behavior change.

Memory aid: Attack = episode; disorder = recurring pattern + aftermath.

2. Differentiate obsession from compulsion and give one example of each. (2mks)

- Obsession: recurrent intrusive unwanted thought/urge/image, e.g. fear of contamination.
- Compulsion: repetitive behavior/mental act to reduce distress, e.g. repeated hand washing.

Memory aid: O = thought; C = conduct.

3. State the four D criteria commonly used when considering abnormality. (2mks)

- Deviance
- Distress
- Dysfunction
- Danger

Memory aid: 4D: Deviance, Distress, Dysfunction, Danger.

4. Define substance use disorder in practical clinical terms. (2mks)

- Problematic pattern of substance use causing significant impairment or distress.
- Look for loss of control, social/role impairment, risky use and/or tolerance/withdrawal.

Memory aid: 4C: Control loss, Consequences, Craving, Continued use.

5. Give two reasons early intervention in substance use disorder is important. **(2mks)**

- Reduces medical, social, occupational and legal harm and risk of escalation.
- Improves opportunity for engagement, relapse prevention, family support and recovery before consequences accumulate.

Memory aid: *EARLY = less Entrenchment, Address Risk, Link help, Yield recovery.*

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. Which psychotherapy concept is most accurately described by the following: the client says whatever thoughts, feelings, images or memories come to mind without censoring them?

- A. Cognitive restructuring
- B. Rational Emotive Behavior Therapy (REBT)
- C. Operant conditioning

D. Free association

Answer: D - Free association. Free association is a classic psychoanalytic technique intended to reduce censorship and reveal unconscious material.

Memory aid: *Psychoanalytic FIRT: Free association, Interpretation, Resistance, Transference.*

2. Which psychotherapy concept is most accurately described by the following: the therapist explains or offers meaning about patterns, defenses, dreams, resistance or behavior?

- A. Person-centered therapy
- B. Interpretation**
- C. Social modeling
- D. Empty-chair technique

Answer: B - Interpretation. Interpretation is the analyst's attempt to make latent or previously unconscious meaning more understandable to the client.

Memory aid: *Interpretation = therapist offers possible meaning.*

3. Which psychotherapy concept is most accurately described by the following: the client unconsciously avoids material or processes that threaten to bring unacceptable conflict into awareness?

- A. Free association
- B. Resistance**
- C. Classical conditioning
- D. Adlerian therapy

Answer: B - Resistance. Resistance is the defensive blocking of threatening material or therapeutic work.

Memory aid: *Resistance = psychological brakes.*

4. A therapist working with Patricia uses an intervention characterized by the client redirects feelings, expectations or relationship patterns from important past relationships onto the therapist. Which approach or technique is being used?

- A. Extinction
- B. Rational emotive imagery
- C. Interpretation
- D. Transference**

Answer: D - Transference. Transference originates primarily from the client's earlier relationships and becomes enacted in the therapeutic relationship.

Memory aid: *TRANSference = client TRANSfers old relationship feelings to therapist.*

5. A therapist working with Victor uses an intervention characterized by the therapist's emotional reactions to a client, especially reactions influenced by the therapist's own history or unresolved issues. Which approach or technique is being used?

- A. Cognitive restructuring
- B. Extinction
- C. Person-centered therapy
- D. Countertransference**

Answer: D - Countertransference. Countertransference concerns the therapist's responses to the client and must be monitored through self-awareness and supervision.

Memory aid: *COUNTER = counselor side of the relational reaction.*

6. A therapist working with Rose uses an intervention characterized by therapy emphasizing unconscious conflict, early relationships, defenses, transference, resistance and insight. Which approach or technique is being used?

- A. Dialectical Behavior Therapy (DBT)
- B. Person-centered therapy
- C. Empty-chair technique

D. Psychoanalytic therapy

Answer: D - Psychoanalytic therapy. Psychoanalytic approaches seek insight into unconscious patterns and how early relationships repeat in present life and therapy.

Memory aid: *FIRT: Free association, Interpretation, Resistance, Transference.*

7. A therapist working with Victor uses an intervention characterized by a goal-directed, socially embedded approach emphasizing conscious choice, responsibility, lifestyle and movement from inferiority toward useful belonging. Which approach or technique is being used?

- A. Social skills training

B. Adlerian therapy

- C. Systematic desensitization
- D. Cognitive behavioral therapy (CBT)

Answer: B - Adlerian therapy. Adlerian therapy views behavior as purposeful and socially related, with emphasis on goals, choice and responsibility. This is explicitly reflected in the opening lecture of the fused audio.

Memory aid: *GSCR: Goal-directed, Social relatedness, Conscious choice, Responsibility.*

8. A therapist working with Fidel uses an intervention characterized by a non-directive relationship based on accurate empathy, unconditional positive regard and congruence. Which approach or technique is being used?

- A. In vivo exposure
- B. Social modeling

C. Person-centered therapy

- D. Progressive muscle relaxation

Answer: C - Person-centered therapy. Rogers' person-centered therapy relies on core facilitative conditions rather than therapist-imposed solutions.

Memory aid: *EUC: Empathy, Unconditional positive regard, Congruence.*

9. In a session with Esther, the counselor emphasizes a structured approach linking thoughts, emotions and behaviors and using cognitive and behavioral change strategies. This BEST illustrates:

- A. Negative reinforcement
- B. Gestalt therapy
- C. Cognitive behavioral therapy (CBT)**
- D. Dialectical Behavior Therapy (DBT)

Answer: C - Cognitive behavioral therapy (CBT). CBT targets maladaptive thinking and behavior through collaborative, structured methods such as cognitive restructuring and behavioral experiments.

Memory aid: *TEA: Thoughts - Emotions - Actions.*

10. Which psychotherapy concept is most accurately described by the following: identifying inaccurate or dysfunctional thoughts, examining evidence and developing more balanced alternatives?

- A. Operant conditioning
- B. Cognitive restructuring**
- C. Extinction
- D. Rational Emotive Behavior Therapy (REBT)

Answer: B - Cognitive restructuring. Cognitive restructuring is a core CBT method for testing and modifying distorted or unhelpful cognitions.

Memory aid: *Catch it, Check it, Change it.*

11. In a session with Victor, the counselor emphasizes Albert Ellis' approach that identifies irrational beliefs between events and emotional consequences and actively disputes those beliefs. This BEST illustrates:

- A. Exposure therapy
- B. Rational Emotive Behavior Therapy (REBT)**
- C. Empty-chair technique
- D. Person-centered therapy

Answer: B - Rational Emotive Behavior Therapy (REBT). REBT uses the ABC model and disputation to replace rigid irrational beliefs with more rational alternatives.

Memory aid: ABCDE: Activating event, Belief, Consequence, Dispute, Effective new belief.

12. A therapist working with Martha uses an intervention characterized by an REBT imagery exercise in which the client vividly imagines a difficult event, experiences the emotion, then practices changing unhealthy emotion by changing beliefs. Which approach or technique is being used?

- A. Systematic desensitization
- B. Countertransference
- C. Rational emotive imagery**
- D. Cognitive behavioral therapy (CBT)

Answer: C - Rational emotive imagery. Rational emotive imagery is an experiential REBT technique for rehearsing healthier emotional responses through belief change.

Memory aid: Imagine - feel - dispute - re-feel.

13. Which psychotherapy concept is most accurately described by the following: a here-and-now experiential approach emphasizing awareness, contact, unfinished business and integration?

- A. Adlerian therapy
- B. In vivo exposure
- C. Gestalt therapy**
- D. Cognitive behavioral therapy (CBT)

Answer: C - Gestalt therapy. Gestalt therapy focuses on present awareness and experiments that help clients own and integrate experience.

Memory aid: Gestalt = awareness in the NOW.

14. A therapist working with Owen uses an intervention characterized by the client speaks to an imagined person or part of self placed in an empty chair, often switching roles to deepen awareness. Which approach or technique is being used?

- A. Gestalt dream work
- B. Empty-chair technique**
- C. Extinction
- D. Rational Emotive Behavior Therapy (REBT)

Answer: B - Empty-chair technique. The empty-chair experiment is associated with Gestalt work on unfinished business, polarities and present-moment experience.

Memory aid: Empty chair = make the absent relationship present.

15. In a session with Wendy, the counselor emphasizes the client re-enacts a dream in the present and may speak as different people, objects or elements of the dream. This BEST illustrates:

- A. Progressive muscle relaxation
- B. Gestalt dream work**
- C. Transference
- D. Classical conditioning

Answer: B - Gestalt dream work. Rather than imposing a fixed interpretation, Gestalt dream work invites the client to own and experience parts of the dream.

Memory aid: Dream it, become it, speak it.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. Accurate empathy, unconditional positive regard and congruence are core person-centered conditions.

These are the classic Rogers conditions emphasized in person-centered therapy.

2. TRUE. Albert Ellis is the major proponent of Rational Emotive Behavior Therapy.

Ellis developed REBT.

3. TRUE. Sigmund Freud is the foundational proponent of psychoanalytic therapy.

Freud developed psychoanalysis.

4. FALSE. Free association requires the therapist to censor embarrassing client material.

The client is invited to say what comes to mind without censorship.

5. FALSE. Transference refers mainly to the therapist transferring personal feelings onto the client.

That description is closer to countertransference; transference is the client carrying past relationship patterns into the therapeutic relationship.

SECTION C: MODEL SHORT ANSWERS

1. Describe the empty-chair technique. (2mks)

- Place an empty chair as if a significant person or part of self were present.

- Client speaks directly to that chair and may switch roles, exploring unfinished feelings in the here-and-now.

Memory aid: *Empty chair makes the absent dialogue present.*

2. Describe progressive muscle relaxation. (2mks)

- Systematically tense a muscle group briefly while noticing tension.

- Release and notice relaxation, then proceed through major muscle groups with calm breathing.

Memory aid: *Tense - notice - release.*

3. Define free association. (1mks)

- Client verbalizes whatever comes to mind without censorship, however trivial or uncomfortable, to help reveal patterns and unconscious material.

Memory aid: *Free means uncensored.*

PART THREE: ETHICAL/LEGAL - ANSWERS

1. A counselor asks which principle or professional concept is defined by respecting the client's right to make informed choices about treatment. The BEST answer is:

- A. Subpoena response
- B. Autonomy**
- C. Gift decision-making
- D. Beneficence

Answer: B - Autonomy. Autonomy supports informed choice, consent and respect for the client's self-determination.

Memory aid: A = Allow informed choice.

2. A counselor asks which principle or professional concept is defined by acting to promote the client's welfare and benefit. The BEST answer is:

- A. Subpoena response
- B. Sexual relationship with a current client
- C. Beneficence**
- D. Confidentiality

Answer: C - Beneficence. Beneficence asks the professional to actively contribute to the client's well-being.

Memory aid: BENEFIT = beneficence.

3. A counselor asks which principle or professional concept is defined by avoiding or minimizing harm. The BEST answer is:

- A. Nonmaleficence**
- B. Ethical response to colleague misconduct
- C. Record keeping
- D. Mandated reporting

Answer: A - Nonmaleficence. Nonmaleficence is the duty to do no harm and to avoid exposing clients to unnecessary risk.

Memory aid: NON-maleficence = NO harm.

4. In professional practice, a situation calls for fair and equitable distribution of services, benefits, burdens and opportunities. Which ethical/legal concept is MOST directly involved?

- A. Duty to warn/protect
- B. Sexual relationship with a current client
- C. Fidelity
- D. Justice**

Answer: D - Justice. Justice concerns fairness and equitable treatment, including access and resource allocation.

Memory aid: Justice = fairness.

5. During supervision, Zachary describes truthfulness and honest communication with clients. Which concept should guide the discussion?

- A. Record keeping
- B. Beneficence
- C. Boundary violation
- D. Veracity**

Answer: D - Veracity. Veracity requires accurate, truthful information rather than deception.

Memory aid: VERACITY = verifiable truth.

6. In professional practice, a situation calls for keeping commitments, maintaining trust and being faithful to professional responsibilities. Which ethical/legal concept is MOST directly involved?

- A. Record keeping
- B. Boundary violation
- C. Ethical response to colleague misconduct

D. Fidelity

Answer: D - Fidelity. Fidelity concerns loyalty, promises and trust within professional relationships.

Memory aid: *Fidelity = faithful to promises.*

7. During supervision, Hassan describes an ongoing process in which the client receives understandable information about services, risks, benefits, alternatives, limits of confidentiality and choice. Which concept should guide the discussion?

A. Duty to warn/protect

B. Informed consent

C. Gift decision-making

D. Autonomy

Answer: B - Informed consent. Informed consent is not merely a signature; it is an ongoing communication process supporting voluntary, informed participation.

Memory aid: *BRALC: Benefits, Risks, Alternatives, Limits, Choice.*

8. A counselor asks which principle or professional concept is defined by the professional duty to protect information learned in the therapeutic relationship, subject to recognized legal and ethical exceptions. The BEST answer is:

A. Confidentiality

B. Autonomy

C. Malpractice

D. Ethical response to colleague misconduct

Answer: A - Confidentiality. Confidentiality protects client information but is not absolute; limits should be explained early.

Memory aid: *Confidentiality = keep private, know the exceptions.*

9. A counselor asks which principle or professional concept is defined by the client's broader right to control access to self, body and personal information. The BEST answer is:

A. Beneficence

B. Privacy

C. Technology ethics

D. Confidentiality

Answer: B - Privacy. Privacy is the client's right to be left alone or control access, whereas confidentiality is the professional's duty regarding information received.

Memory aid: *Privacy belongs to the person; confidentiality binds the professional.*

10. In professional practice, a situation calls for a legal protection that may prevent compelled disclosure of certain therapy communications in judicial proceedings, depending on jurisdiction. Which ethical/legal concept is MOST directly involved?

A. Boundary violation

B. Privileged communication

C. Subpoena response

D. Veracity

Answer: B - Privileged communication. Privilege is a legal evidentiary concept and is distinct from the broader ethical duty of confidentiality.

Memory aid: *Privilege = courtroom/legal shield.*

11. During supervision, Hassan describes taking reasonable protective action when a client presents a serious, credible threat toward an identifiable person, as required by applicable law and ethics. Which concept should guide the discussion?

A. Veracity

B. Nonmaleficence

C. Duty to warn/protect

D. Fidelity

Answer: C - Duty to warn/protect. The course tests duty to warn/protect as an exception to confidentiality when serious danger to others is present. Actual legal requirements vary by jurisdiction.

Memory aid: *TIP: Threat serious, Identifiable target, Protect reasonably.*

12. In professional practice, a situation calls for reporting suspected abuse or other legally reportable situations to designated authorities when law or professional duty requires it. Which ethical/legal concept is MOST directly involved?

- A. Veracity
- B. Mandated reporting**
- C. Dual or multiple relationship
- D. Boundary crossing

Answer: B - Mandated reporting. Mandated reporting is a recognized limit to confidentiality; therapists should explain this limit during informed consent.

Memory aid: *Report when law makes reporting mandatory.*

13. In professional practice, a situation calls for professional negligence in which a duty is breached and the breach causes harm. Which ethical/legal concept is MOST directly involved?

- A. Malpractice**
- B. Veracity
- C. Boundary violation
- D. Mandated reporting

Answer: A - Malpractice. Malpractice requires more than an unfavorable outcome: there must be a professional duty, breach, harm and causal link.

Memory aid: *DBHC: Duty, Breach, Harm, Causation.*

14. In professional practice, a situation calls for practicing only within areas supported by appropriate education, training, supervised experience and current skill. Which ethical/legal concept is MOST directly involved?

- A. Gift decision-making
- B. Professional competence**
- C. Veracity
- D. Ethical response to colleague misconduct

Answer: B - Professional competence. Using techniques without adequate training can expose clients to harm and the professional to ethical or legal liability.

Memory aid: *Know your scope; seek supervision or refer.*

15. In professional practice, a situation calls for a situation in which the therapist has another significant role or relationship with the client in addition to therapy. Which ethical/legal concept is MOST directly involved?

- A. Dual or multiple relationship**
- B. Record keeping
- C. Nonmaleficence
- D. Privacy

Answer: A - Dual or multiple relationship. Multiple relationships are not automatically harmful in every context, but should be avoided or carefully managed when they risk exploitation, impaired objectivity or harm.

Memory aid: *Two roles = review power, objectivity, exploitation and harm.*

SECTION B: MODEL SHORT ANSWERS

1. What does duty to warn/protect mean in therapy? (2mks)

- When a serious credible threat to an identifiable person is present, the therapist may have to take reasonable protective action according to applicable law and ethics, even though confidentiality is normally protected.

Memory aid: *TIP: Threat, Identifiable person, Protect.*

2. Define mandated reporting. (1mks)

- A legal/professional requirement to report specified situations such as suspected abuse to designated authorities; it is a limit to confidentiality.

Memory aid: *Mandatory = must report when the law says so.*

3. State two ways a therapist should manage sexual or romantic feelings toward a client. **(1mks)**

- Acknowledge and monitor the feelings rather than act on them.
- Seek supervision/consultation and strengthen boundaries; refer if objectivity/competence cannot be maintained.

Memory aid: FEEL, do not ACT: notice, supervise, protect boundaries.

4. What should a therapist do when credible serious unethical behavior by a colleague is suspected? **(1mks)**

- Clarify facts, consult the code/supervisor as appropriate, protect the client and follow formal reporting procedures when required.

Memory aid: CARE: Consult, Assess, Report, Ensure safety.

SECTION C: MODEL SHORT ESSAYS

1. Differentiate privacy from confidentiality. **(2mks)**

- Privacy is the client's right to control access to self and personal information.
- Confidentiality is the professional duty to protect information obtained in the professional relationship.

Memory aid: Privacy = person's right; confidentiality = professional's duty.

2. Differentiate confidentiality from privileged communication. **(2mks)**

- Confidentiality is an ethical/professional duty.
- Privilege is a legal evidentiary protection against compelled disclosure in certain proceedings and depends on jurisdiction.

Memory aid: Privilege = courtroom shield.

3. Define malpractice. **(1mks)**

- Professional negligence in which a duty is breached and the breach causes harm; commonly considered through duty, breach, causation and damages/harm.

Memory aid: DBHC.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. Which counseling/group/cultural concept is BEST described by: ongoing development of awareness, cultural knowledge, clinical skills and meaningful encounters with diverse clients?

- A. Performing stage
- B. Skills-development group
- C. Termination and follow-up

D. Cultural competence

Answer: D - Cultural competence. Cultural competence requires reflective awareness plus knowledge, skills and real encounters; it is not achieved by assuming everyone should conform to the therapist's culture.

Memory aid: AKSE: Awareness, Knowledge, Skills, Encounters.

2. In counseling practice, Samuel encounters a situation involving judging other cultures primarily by the standards of one's own culture and treating one's own as the norm. Which concept BEST applies?

A. Ethnocentrism

- B. Termination and follow-up
- C. Active listening
- D. Psychoeducational group

Answer: A - Ethnocentrism. Ethnocentrism places one's own cultural frame at the center and may distort assessment or communication.

Memory aid: ETHNO-centred = my culture at the centre.

3. Which counseling/group/cultural concept is BEST described by: pressuring a client to adopt the therapist's or dominant culture's beliefs, values or practices?

- A. Psychoeducational group
- B. Goal setting
- C. Skills-development group

D. Cultural imposition

Answer: D - Cultural imposition. Cultural imposition occurs when the helper pushes their own values or practices onto the client.

Memory aid: IMPOSITION = impose my culture.

4. In counseling practice, Esther encounters a situation involving using a qualified interpreter rather than relying on a child or untrained friend when language barriers affect care. Which concept BEST applies?

A. Professional interpreter use

- B. Psychoeducational group
- C. Cultural competence
- D. Support group

Answer: A - Professional interpreter use. Qualified interpreters improve accuracy, confidentiality and informed consent.

Memory aid: Interpreter = accuracy + confidentiality.

5. A trainee observes a group emphasizing mutual support, shared experience, acceptance and coping among people facing similar problems. This is an example of:

- A. Rapport building
- B. Support group**
- C. Ethnocentrism
- D. Assessment

Answer: B - Support group. Support groups reduce isolation and offer mutual aid through shared experience.

Memory aid: SUPPORT = Similar people Understanding and Providing Peer help.

6. A trainee observes a structured group focused on teaching information, understanding a condition and coping knowledge. This is an example of:

- A. Case formulation
- B. Assessment
- C. Performing stage

D. Psychoeducational group

Answer: D - Psychoeducational group. Psychoeducational groups primarily teach information and understanding; they may include discussion and practice.

Memory aid: *EDU = education is central.*

7. In counseling practice, Patricia encounters a situation involving a group focused on learning and practicing specific behavioral or interpersonal skills. Which concept BEST applies?

- A. Norming stage
- B. Treatment plan
- C. Skills-development group**
- D. Professional interpreter use

Answer: C - Skills-development group. Skills groups emphasize rehearsal, practice and feedback rather than information alone.

Memory aid: *SKILLS = Show, Keep practicing, Improve, Learn, feedback.*

8. Which counseling/group/cultural concept is BEST described by: a structured group using cognitive and behavioral methods such as thought monitoring, skills practice and behavioral exercises?

- A. Cognitive-behavioral group**
- B. Storming stage
- C. Treatment plan
- D. Psychoeducational group

Answer: A - Cognitive-behavioral group. CBT groups combine structured learning with practice and feedback around thoughts and behaviors.

Memory aid: *Group CBT = Think + Do together.*

9. Which counseling/group/cultural concept is BEST described by: a recurrent functional position adopted by a member, such as scapegoat, monopolizer, peacemaker or spokesperson?

- A. Group role**
- B. Goal setting
- C. Therapeutic confrontation
- D. Group confidentiality

Answer: A - Group role. Group roles describe patterned ways members participate and influence group dynamics.

Memory aid: *Role = repeated position in the group.*

10. Which counseling/group/cultural concept is BEST described by: an agreement that members protect information disclosed by other members, while the leader explains that peer confidentiality cannot be guaranteed in the same way as professional confidentiality?

- A. Psychoeducational group
- B. Group confidentiality**
- C. Termination and follow-up
- D. Ethnocentrism

Answer: B - Group confidentiality. Confidentiality is essential in groups, but leaders should explain realistic limits because members are not all licensed professionals.

Memory aid: *What is said in group should stay in group - with limits explained.*

11. In counseling practice, Tina encounters a situation involving early group stage characterized by orientation, uncertainty, dependence and testing. Which concept BEST applies?

- A. Forming stage**
- B. Cultural competence

- C. Intervention stage
- D. Norming stage

Answer: A - Forming stage. Forming is the beginning stage in which members learn the purpose, structure and safety of the group.

Memory aid: FSNPA: *Forming, Storming, Norming, Performing, Adjourning.*

12. A trainee observes group stage in which conflict, resistance, competition or emotional testing becomes more visible. This is an example of:

- A. Ethnocentrism
- B. Active listening
- C. Support group
- D. Storming stage**

Answer: D - Storming stage. Storming often brings disagreement and challenges to roles or leadership.

Memory aid: *Storm = conflict weather.*

13. In counseling practice, Fidel encounters a situation involving group stage in which cohesion, trust, norms and cooperation consolidate. Which concept BEST applies?

- A. Cultural imposition
- B. Norming stage**
- C. Treatment plan
- D. Psychoeducational group

Answer: B - Norming stage. Norming is marked by clearer expectations and stronger cohesion.

Memory aid: *Norming = norms settle.*

14. Which counseling/group/cultural concept is BEST described by: mature working stage in which members use the group productively toward goals?

- A. Open-ended questioning
- B. Performing stage**
- C. Cultural competence
- D. Termination and follow-up

Answer: B - Performing stage. Performing is the task-focused, therapeutically productive phase.

Memory aid: *Performing = productive work.*

15. In counseling practice, Victor encounters a situation involving ending stage involving review, separation, consolidation and transfer of learning. Which concept BEST applies?

- A. Treatment plan
- B. Cognitive-behavioral group
- C. Adjourning stage**
- D. Termination and follow-up

Answer: C - Adjourning stage. Adjourning addresses termination and helps members carry gains beyond the group.

Memory aid: *Adjourning = goodbye and generalize gains.*

SECTION B: MODEL SHORT ESSAYS

1. Outline two benefits of group therapy. (4mks)

- Reduces isolation/universality: members learn they are not alone.
- Provides mutual support and feedback.
- Allows interpersonal learning, modeling and practice.
- Can be efficient and increase access - any two well explained.

Memory aid: U-S-M: *Universality, Support, Modeling.*

2. State two characteristics of a culturally competent therapist. (4mks)

- Self-awareness of own values/biases.
- Knowledge of different cultural contexts and humility about what is not known.
- Adaptable communication/clinical skills.
- Willingness to seek cultural encounters/consultation and avoid imposition - any two explained.

Memory aid: AKSE.

3. What is the counseling process? (2mks)

- A collaborative sequence that typically includes engagement/rapport, assessment, goal setting, intervention, review, and termination/follow-up.

Memory aid: RAGIRT: Rapport, Assessment, Goals, Intervention, Review, Termination.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 01	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 01

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Work stress and first counseling session - anchor: First-session counseling process with person-centered engagement plus problem-focused assessment

Jocelyn, 31, works as a nurse and has recently been working 11-12 hour days. she has become irritable with colleagues, sleeps poorly, has stopped seeing friends and says, "I am losing control of my life." Jocelyn has never attended counseling before and is unsure what to expect.

Model response / marking points (25 marks):

1. Establish rapport, privacy and a respectful therapeutic setting.
2. Explain role, informed consent and limits of confidentiality.
3. Use open questions, active listening, paraphrase, reflection and summarizing.
4. Assess stressors, sleep, functioning, substance use, supports, medical/mental-health history and risk including suicide/self-harm.
5. Identify strengths and the client's own priorities.
6. Agree on a preliminary formulation: overload, reduced recovery, irritability and impaired balance; avoid premature diagnosis.
7. Set one or two SMART initial goals.
8. Introduce an appropriate first coping intervention such as breathing/relaxation, activity scheduling, boundaries or problem solving.
9. Agree on homework/self-monitoring and next appointment.
10. Close by summarizing, checking understanding and inviting questions.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: RAPPORT: Rapport, Agreement/consent, Problem assessment, Prioritize risk, Objectives, Response/technique, Take-home/follow-up.

2. Driving phobia - anchor: Behavior therapy - graded exposure/systematic desensitization, with CBT elements as needed

Rose, 38, stopped driving after a serious road accident many years ago. As a passenger he develops sweating, shaking, light-headedness and an urge to escape, except when a trusted relative drives and talks calmly. A new job now requires independent travel and Rose wants to regain driving confidence.

Model response / marking points (25 marks):

1. Assess the fear, accident memory, panic symptoms, avoidance, current driving competence and safety.
2. Explain the fear-avoidance cycle and rationale for exposure.
3. Teach a coping/relaxation skill without turning it into avoidance.
4. Build a hierarchy from low to high fear (e.g., sitting in parked car, engine on, empty road, short familiar route, busier road).
5. Use repeated in vivo graded exposure, remaining in each step long enough for learning.

6. Reduce safety behaviors gradually while keeping actual road safety.
7. Identify and test catastrophic thoughts such as "I will certainly crash" or "anxiety means danger."
8. Use homework and record anxiety/predictions/outcomes.
9. Reinforce progress and move up the hierarchy collaboratively.
10. Plan relapse prevention and future independent practice.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: RHE: Relax/coping, Hierarchy, Exposure - then Repeat and Review.

SECTION II

1. Depression after loss - anchor: Major depressive presentation - CBT with safety assessment and behavioral activation

For the past three weeks, Joyce, 45, has felt sad nearly every day after losing a job. she reports loss of interest, fatigue, poor concentration, reduced appetite, insomnia and thoughts such as "I am useless and nothing will improve."

Model response / marking points (25 marks):

1. Recognize a depressive syndrome and assess duration, severity, impairment and differentials.
2. Ask directly about suicidal thoughts, plan, intent, means and protective factors; escalate/referral if risk is acute.
3. Collaboratively map situation-thought-emotion-behavior links.
4. Identify distortions such as overgeneralization, labeling and hopeless predictions.
5. Use evidence for/against and balanced alternative thoughts.
6. Introduce graded behavioral activation beginning with small achievable activities.
7. Address sleep/routine and social support.
8. Set SMART goals and a simple monitoring sheet.
9. Coordinate medical/psychiatric assessment when severity, suicidality or medication questions require it.
10. Review learning and homework.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: CBT TEA + SIGECAPS; safety first.

2. Bereavement and person-centered counseling - anchor: Person-centered therapy with grief assessment

Thomas, 52, recently lost a spouse and arrives tearful, disheveled and unsure whether the intensity of grief is "normal." he says family members keep telling him to "move on," which has made him stop talking about the loss.

Model response / marking points (25 marks):

1. Create safety and let the client set the pace.
2. Use accurate empathy and reflective listening.
3. Offer unconditional positive regard/nonjudgment.
4. Be congruent/genuine without making the session about the therapist.
5. Invite the client to describe the meaning of the loss and present emotions.
6. Avoid premature advice, reassurance or forced positivity.
7. Assess functioning, supports and safety, including suicidal ideation where indicated.
8. Differentiate normal grief from a depressive disorder through duration, pervasive anhedonia/hopelessness and functional impairment without pathologizing grief automatically.
9. Help the client identify needs and supports.

10. Summarize and collaboratively decide next steps.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *EUC: Empathy, Unconditional positive regard, Congruence.*